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<Reference: 97423085-FA> 29 July 2025

Urgent Field Safety Notice - Product Advisory
WATCHMAN TruSeal™ Access System, WATCHMAN FXD Curve™ Access System

Dear «Users_Name»,

This letter provides important information related to the risk of air embolism during WATCHMAN™ procedures as detailed in Appendix 1.

This notification is in response to observations of air embolism complaints received by Boston Scientific related to WATCHMAN procedures. The global rate of reported air embolism under any sedation type during the WATCHMAN procedure is 0.06%, associated with death in 0.009% of cases. After a comprehensive investigation, it was concluded that these complaints are not associated with the design or manufacture of the WATCHMAN system. Based on the review of the complaints, Boston Scientific has identified a higher likelihood of air embolism events when procedures are performed without positive pressure-controlled ventilation. According to published literature and clinical data [1-9], in percutaneous procedures requiring transseptal access to the left atrium when conscious or deep sedation is used, patients have an approximately three-times higher risk (clinical reference 3, U.S. study) of negative left atrium pressure and air ingress. This risk is especially prevalent in patients with pre-existing low left atrial pressure, hypovolemia, and partial upper airway collapse.

The WATCHMAN Access Systems Instructions for Use (IFUs) and WATCHMAN physician training will be updated to emphasize instructions related to Access System air management. This update will strengthen the information provided to clinicians regarding the potential for air embolism during WATCHMAN procedures performed under conscious or deep sedation and provide potential mitigation strategies.

Our records indicate that your facility received some of the concerned product. **The table below provides a complete list of all affected products**, including Material Number (UPN), Material Description, GTIN and Batch number. Please note that **only the devices listed below are affected. No other Boston Scientific product is involved in this Field Safety Notice**.
Boston Scientific is not removing any affected devices from the field, and they all remain available for use.

Material	Material Description	GTIN	Batch
M635TS70010	WATCHMAN TruSeal Access System SGL, OUS	08714729965732	All non-expired batches
M635TS70020	WATCHMAN TruSeal Access System DBL, OUS	08714729965749	
M635TS70040	WATCHMAN TruSeal Access System ANT, OUS	08714729965756	
M635TS80010	WATCHMAN FXD Curve Access Sys Sgl, OUS	00191506013820	
M635TS80020	WATCHMAN FXD Curve Access Sys Dbl, OUS	00191506013837	

Clinical Impact

The risk of air embolism is inherent to any percutaneous procedure requiring transseptal access to the left atrium, including WATCHMAN procedures. The current IFU includes instructions to flush devices and to introduce or remove devices slowly, which effectively mitigate the risk of air embolism for procedures performed under general anesthesia.

According to this investigation, during WATCHMAN procedures performed under conscious or deep sedation, the most frequent clinical manifestation of air embolism was ST-segment elevation or visible air bubbles during imaging. Most of these events either resolved on their own or required temporary medical intervention. **During procedures performed without positive pressure ventilation, there is a known risk of air embolism leading to severe outcomes, including life-threatening or fatal consequences.** These outcomes have been observed in this investigation and include arrhythmia, hemodynamic collapse, stroke (CVA), or other end-organ failure caused by ischemia.

The risk of air embolism is acute in nature and limited to the duration of the implant procedure. Patients who have a previously implanted WATCHMAN device do not require additional patient management and should continue to follow standard patient care at the discretion of their physician.

Instructions:

1- **Please read carefully the Field Safety Notice letter and IFU updates related to air embolism as detailed in Appendix 1. These updates will be added to the WATCHMAN Access Systems IFUs and WATCHMAN Physician Training. Immediately post this information in a visible location near the product to ensure this information is easily accessible to all users of the device.**

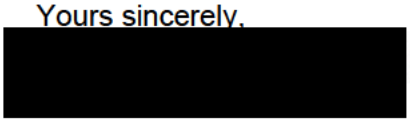
2- **Please complete the attached Acknowledgement Form even if you do not have any affected product**.

3- **When completed, please return the Acknowledgement Form to your Boston Scientific office for the attention of «Customer_Service_Fax_Number» on or before 29 August 2025**.

4- Please pass on this notice to any healthcare professional from your organization that need to be aware and to any organization where the potentially affected devices have been transferred (if appropriate). Please provide Boston Scientific with details of any affected devices that have been transferred to other organizations (if appropriate).

Your national Competent Authority has been informed of this Field Safety Notice.
Any adverse events or quality concerns associated with use of this device should be reported to Boston Scientific via email at ICardioQAComplaints@bsci.com.

We regret any inconvenience that this action may cause, and we appreciate your understanding as we act to ensure patient safety and customer satisfaction.
If you have any questions or would like assistance with this Field Safety Notice, please contact your local Sales Representative.
Yours sincerely,



Vice President, Quality Assurance
Boston Scientific

Attachment: - Acknowledgement Form

Appendix 1- Pending IFU and Physician Training Updates

The following warning and precaution statements are planned to be added in the IFUs:

Warning:

Patients under deep or conscious sedation are at increased risk for negative left atrial pressures, especially in the presence of hypovolemia or partial upper airway obstruction. Patients under controlled positive pressure ventilation such as general anesthesia have a reduced risk of negative atrial pressures. Negative left atrial pressures may increase the risk of air ingress through the hemostasis valve, particularly when the valve is open while introducing, removing or exchanging devices, which may result in air embolism. The most appropriate anesthesia method should be based on individual patient characteristics. Additional caution should be used with patients under deep or conscious sedation, such as:

- Ensure the patient is not hypovolemic,
- Make device exchanges with the access system valve below the level of the heart or under fluid, and withdraw devices slowly until near the valve, then make exchanges during expiration.

Precaution: Patients should not be hypovolemic, particularly if not under positive pressure ventilation, to reduce the likelihood of negative left atrial pressures and air embolism.

Precaution: Hold the WATCHMAN access sheath valve below heart level and/or under fluid during insertion to reduce the likelihood of air ingress. Vacuum can be minimized by withdrawing devices slowly until near the valve, then making exchanges during expiration.

Appendix 2- Clinical References

1. Franzen OW, Klemm H, Hamann F, et al. Mechanisms underlying air aspiration in patients undergoing left atrial catheterization. Catheter Cardiovasc Interv. Mar 1 2008;71(4):553-8. doi:10.1002/ccd.21445
2. Kuwahara T, Takahashi A, Takahashi Y, et al. Clinical characteristics of massive air embolism complicating left atrial ablation of atrial fibrillation: lessons from five cases. Europace. Feb 2012;14(2):204-8. doi:10.1093/europace/eur314
3. Kapadia SR, Yeh RW, Price MJ, et al. Outcomes With the WATCHMAN FLX in Everyday Clinical Practice From the NCDR Left Atrial Appendage Occlusion Registry. Circ Cardiovasc Interv. Sep 2024;17(9):e013750. doi:10.1161/CIRCINTERVENTIONS.123.013750
4. Kawaguchi N, Suzuki A, Usui M, et al. Clinical Effect of Adaptive Servo-Ventilation on Left Atrial Pressure During Catheter Ablation in Sedated Patients With Atrial Fibrillation. Circ J. Jul 21 2021;85(8):1321-1328. doi:10.1253/circj.CJ-20-1263
5. Ikoma T, Naruse Y, Kaneko Y, et al. Prevalence and Characteristics of Inspiration-Induced Negative Left Atrial Pressure during Pulmonary Vein Isolation. J Cardiovasc Dev Dis. Feb 26 2023;10(3)doi:10.3390/jcdd10030101
6. Miyazaki S, Hasegawa K, Mukai M, et al. Clinically manifesting air embolisms in cryoballoon ablation: Can novel water buckets reduce the risk? Innovations in Clinical Electrophysiology. 2020;6(9):1067-72.
7. Saw J, Holmes DR, Cavalcante JL, et al. SCAI/HRS Expert Consensus Statement on Transcatheter Left Atrial Appendage Closure. J Soc Cardiovasc Angiogr Interv. May-Jun 2023;2(3):100577. doi:10.1016/j.jscai.2022.100577
8. Piayda K, Hellhammer K, Nielsen-Kudsk JE, et al. Clinical outcomes of patients undergoing percutaneous left atrial appendage occlusion in general anaesthesia or conscious sedation: data from the prospective global Amplatzer Amulet Occluder Observational Study. BMJ Open. Mar 24 2021;11(3):e040455. doi:10.1136/bmjopen-2020-040455
9. McCarthy CJ, Behravesh S, Naidu SG, Oklu R. Air Embolism: Practical Tips for Prevention and Treatment. J Clin Med. Oct 31 2016;5(11)doi:10.3390/jcm5110093

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Please complete the form & Send it to:
«Customer_Service_Fax_Number»

«Sold_To» - «Hospital_Name» - «City» - «Country_name»

Acknowledgement Form – Product Advisory

WATCHMAN TruSeal™ Access System, WATCHMAN FXD Curve™ Access System

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By signing this form, I confirm that

I have read and understood
the Boston Scientific Field Safety Notice

dated 29 July 2025 for the

WATCHMAN TruSeal™ Access System,
WATCHMAN FXD Curve™ Access System

NAME* _____ Title _____

Telephone _____ Email _____

Customer' **SIGNATURE*** _____ **DATE*** _____
* Required field dd/mm/yyyy